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PERSPECTIVE

ADHD Across the Lifespan: Reconsidering Assumptions of Diagnostic Permanence

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ABSTRACT

Attention-deficit/hyperactivity disorder (ADHD) is increasingly diagnosed across the lifespan, yet there is a growing tendency to treat diagnoses as permanently fixed once assigned. Diagnosis requires not only the presence of characteristic symptoms but also clinically significant functional impairment that is most appropriately explained by ADHD, rather than another condition. Longitudinal evidence demonstrates that ADHD trajectories are heterogeneous and non-linear, with fluctuating symptom expression and variable relationships to functional impairment. Rather than conceptualising an ADHD diagnosis as permanently fixed, greater recognition of this complexity supports longitudinal clinical formulation and more adaptive biopsychosocial care across the lifespan.

JEL Classification: Pediatric medicine, General medicine, Mental disorders, Health services administration, Ethics and law

1 | Introduction

Recent discussion within the *Medical Journal of Australia* has appropriately examined concerns regarding commercialisation, diagnostic inflation and the rapid expansion of adult attention-deficit/hyperactivity disorder (ADHD) assessment models [1, 2]. Less attention, however, has been paid to a related conceptual issue: the increasing tendency for an ADHD diagnosis to be understood as a permanently fixed state once assigned.

2 | Functional Impairment

ADHD is conceptualised as a neurodevelopmental disorder characterised by persistent patterns of inattention and/or hyperactivity–impulsivity associated with clinically significant functional impairment across different settings. In practical terms, this may include missed deadlines, failure to progress at work or school, repeated relationship breakdowns or an inability to manage everyday tasks, rather than simply reflecting a different style of thinking or working. Importantly, diagnosis requires that symptoms are not better explained by another mental disorder [3]. Thus, a clinician diagnosing ADHD must exercise judgement that ADHD provides the most appropriate explanation for a patient's symptoms and associated impairment at a particular point in time.

Functional impairment is inherently contextual and may vary substantially according to environmental demands, occupational fit, psychosocial supports, developmental stage and broader societal expectations [2, 3]. An individual who experiences marked impairment within one educational or occupational setting may function effectively within another environment better aligned with their attentional profile and cognitive style. Functional impairment may also be less visible in individuals who achieve through substantial compensatory effort or unsustainable levels of cognitive and emotional exertion. In some cases, impairment may manifest not as overt failure, but as chronic exhaustion, disproportionate effort required to maintain expected performance or persistent underachievement relative to cognitive capacity and developmental opportunity.

Broader societal and occupational changes may also influence the extent to which attentional vulnerabilities become

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impairing. Contemporary educational and occupational environments increasingly place sustained demands on executive functioning, multitasking, administrative organisation and prolonged attentional control, potentially altering the threshold at which attentional traits become functionally disabling. Conceptual and practical ambiguities in how functional impairment is defined and operationalised in adult ADHD assessment can therefore have substantial implications for who is diagnosed and how severity is understood [4].

3 | ADHD Across Time

In clinical settings, it is increasingly common for adults presenting for reassessment to describe a prior ADHD diagnosis as definitively establishing that they have ADHD for life, independent of how their current symptoms, functioning or circumstances have changed. There is currently no consensus regarding how diagnostic formulations should be conceptualised or documented when an individual no longer meets the diagnostic criteria [5]. In contrast to depression, for instance, full remission is not explicitly recognised within diagnostic systems and clinical practice: the *Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition* (DSM-5) recognises only partial remission, and the *International Classification of Diseases, 11th Revision* (ICD-11) does not offer a remission code at all. This may be due to the conceptualisation of neurodevelopmental disorders as distinct from psychiatric disorders, with diagnoses often regarded as enduring or permanent. Although ADHD is appropriately classified as a neurodevelopmental disorder, the assumption that a diagnosis necessarily implies a fixed, lifelong disorder is not fully supported by longitudinal evidence [6–8].

Longitudinal studies demonstrate that ADHD trajectories are frequently non-linear and heterogeneous, with fluctuating symptom burden across the lifespan. Follow-up studies have found that a proportion of individuals diagnosed in childhood no longer meet full diagnostic criteria in adulthood, whereas others move through periods of remission and recurrence rather than experiencing stable persistence or stable recovery [6–9]. At the same time, a substantial proportion continue to experience persistent and clinically significant difficulties into adulthood, especially those with more severe childhood symptoms, comorbidity and marked impairment [6–10].

Importantly, remission should not be assumed to invalidate a previous diagnosis. A time-limited or changing presentation must be distinguished from misdiagnosis, where ADHD was never the most accurate explanation for an individual's difficulties. This reflects a broader principle within psychiatry: diagnoses are clinical formulations based on multiple factors, including symptom patterns, functional impairment and context, all of which may evolve with time [11]. Longitudinal assessment is therefore essential.

Recognition of longitudinal variability should also not minimise the potentially profound developmental impact of ADHD during childhood and adolescence, particularly in more severe presentations. Early attentional and executive impairments may limit educational progress, strain peer relationships and undermine the development of emotional regulation and self-confidence during formative years [6, 12]. Consequently, even where overt symptom burden later attenuates or full diagnostic threshold is no longer met, the cumulative effects of earlier impairment may continue to shape adult functioning and life trajectory.

4 | Clinical Implications

Some individuals who no longer meet full diagnostic criteria may continue to experience residual attentional vulnerabilities requiring substantial compensatory effort, with ongoing difficulties in everyday life [12]. Therefore, a nuanced consideration of both symptom persistence and functional impairment may be more clinically informative than binary classifications of ADHD versus non-ADHD [8, 9].

Increasingly, an ADHD diagnosis entails regulatory consequences such as educational accommodations, access to stimulant medications and insurance assessments [5]. For many individuals, an ADHD diagnosis provides a meaningful explanation for long-standing experiences and may become an important part of personal identity [13]. Consequently, diagnostic formulations may become progressively reified over time, reducing openness to reassessment or consideration of alternative explanations for attentional and executive symptoms [3, 14, 15]. This reification is clinically significant: a prior diagnosis can anchor subsequent clinical encounters in ways that discourage the differential thinking that diagnosis itself requires.

Psychiatric diagnosis is better understood as a clinical formulation that evolves alongside the individual across the lifespan. To be clear, this longitudinal approach does not advocate repeated formal diagnostic assessments. Rather, each clinical encounter provides an opportunity to revisit not only whether ADHD symptoms persist, but whether those symptoms continue to produce the clinically significant functional impairment required for diagnosis, refining the formulation accordingly. Recognising that clinical formulation may evolve over time does not diminish the explanatory value or personal meaning an ADHD diagnosis may hold. Rather, it acknowledges that although neurodevelopmental vulnerabilities may persist, the relationship between those vulnerabilities, functional significance and an individual's lived experience may continue to evolve across the lifespan.

This perspective has important implications for treatment. Conceptualising ADHD as permanently fixed may inadvertently encourage overly symptom-centred models of care in which treatment success is primarily defined by pharmacological reduction of attentional symptoms. Accordingly, treatment may be better understood as part of a broader biopsychosocial approach aimed at reducing impairment, strengthening adaptation and supporting individuals to build lives in which attentional vulnerabilities no longer result in clinically significant functional impairment. Pharmacological treatment may remain an important component of care for many individuals, but optimal management may also involve ADHD-informed psychological support, environmental modification, occupational adaptation and attention to person–environment fit over time [3, 14].

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More broadly, the current absence of formal diagnostic recognition of remission should not constrain clinical practice or limit the possibilities considered for individuals living with ADHD. Instead, clinicians should remain open to the possibility that, through treatment, adaptation and changes in person–environment fit, some individuals may no longer experience the clinically significant functional impairment required for diagnosis. Clinical formulation should therefore consider functioning within an individual's current treatment and environmental context, recognising that successful psychological adaptation and environmental supports are themselves important therapeutic outcomes rather than factors to be discounted when considering ongoing diagnosis. This does not imply that individuals who are functioning well should continue to be assessed. Rather, it highlights the importance of avoiding assumptions of diagnostic permanence whenever ADHD is encountered in the clinical context.

Taken together, these considerations suggest a model of care in which diagnosis, formulation and follow-up care are understood as longitudinal processes rather than discrete events. This stance neither undermines the validity of ADHD as a diagnosis nor minimises the significant burden it carries for individuals with persistent impairment; rather, it aligns clinical practice with the longitudinal evidence, recognising that trajectories are heterogeneous, impairment is contextual and formulation should remain responsive to change across the lifespan.

5 | Conclusion

Current ADHD discourse increasingly risks conceptualising an ADHD diagnosis as static and permanent once assigned. ADHD is better understood as a neurodevelopmental condition with heterogeneous developmental trajectories, fluctuating symptom expression and variable relationships to functional impairment over time. Greater recognition of this complexity may support increased emphasis on longitudinal assessment and adaptive biopsychosocial treatment planning across the lifespan.

Conceptualising an ADHD diagnosis as necessarily permanent risks overlooking the dynamic interaction between neurodevelopmental vulnerabilities, treatment, development and environment across the lifespan. Although diagnostic classifications provide an essential framework for clinical practice, they should not constrain clinical thinking or limit the possibilities considered for individuals living with ADHD. Greater recognition of heterogeneous trajectories and the possibility of remission encourage clinicians to remain open to the possibility that, through treatment, adaptation and changes in person–environment fit, some individuals may no longer experience the clinically significant functional impairment required for diagnosis. Although ADHD may profoundly shape a person's early life, it need not define the boundaries of their future.

Author Contributions

Calina Ouliaris: conceptualization, investigation, writing – original draft, writing – review and editing, formal analysis.

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Conflicts of Interest

Calina Ouliaris works as a Consultant Psychiatrist in private practice, which includes assessment and management of ADHD. No other relevant financial relationships are declared. The views expressed in this manuscript are those of the author alone and do not necessarily reflect those of the organisations with which she is affiliated.

Data Availability Statement

No new data were generated or analysed in support of this manuscript.

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